

Salford City Council

Implementation note: consideration of health issues in the determination of planning applications

September 2024

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1. Introduction

- 1.1 This implementation note has been prepared to provide clarity as to how Salford City Council will consider health issues through the consideration of planning applications, particularly in the context of specific policies in the Salford Local Plan: Development Management Policies and Designations, and the sub-regional joint development plan document Places for Everyone.

2. Salford Local Plan: Development Management Policies and Designations

- 2.1 The [Salford Local Plan: Development Management Policies and Designations](#) (SLP:DMP) was adopted in January 2023 and forms part of the statutory development plan for Salford.

- 2.2 Chapter 14 of the SLP:DMP is clear that: “A healthy population is an essential component of a sustainable Salford” (paragraph 14.1). Paragraph 14.4 goes on to state that:

“The Local Plan has a significant role to play in delivering the Salford Locality Plan vision, an age friendly city and supporting health improvements more generally. Health considerations are therefore integrated into all aspects of the plan, for example with it:

- Promoting healthy lifestyles and increased physical activity through the design of new development (chapter 19), by improving the city’s green infrastructure network (chapter 22), enhancing the quality and accessibility of recreation opportunities (chapter 24) and expanding the network of attractive pedestrian and cycling routes (policy A3)
- Reducing contributors to poor health and mitigating their risks, such as those associated with climate change (policy CC1), poor air quality (various policies in the plan including policies PH1 and GI1), flooding (chapter 18), crime (policy D6) and hazardous uses (chapter 25)
- Promoting a stronger local economy (chapter 9), recognising the clear links between income and health
- Improving and creating better access to education, training and employment (policy F2 and chapter 13)
- Supporting good mental health, such as by helping to reduce deprivation (throughout the plan), promoting social interaction through the design of high quality public spaces and places (policy D4), improving access to nature and green spaces (chapters 22-24), and providing decent and affordable homes within an attractive environment (chapter 11)
- Supporting healthy eating by increasing opportunities for growing food (policy R1) and improving the accessibility of retail facilities selling fresh food (chapter 12)”

- 2.3 Policy HH1 of the SLP:DMP specifically relates to development and health.

Implementation of SLP:DMP Policy HH1

Developments in uses A) to D)

- 2.4 For schemes including one or more of the uses listed under points A to D¹, applicants should provide supporting information as part of their submission to demonstrate compliance with policy HH1. This should consider the location of the proposed use and assess whether there are existing concentrations of that use. This is the policy test in policy HH1. Where uses in points A to D have been justified, the applicant will then also be required to demonstrate compliance with the following:

“Where uses identified in points A-D above can be justified, these shall be located away from places that are regularly frequented by younger people such as schools and youth facilities. The proximity to other community uses where the residents can be vulnerable will also be considered, for example refuges and temporary accommodation for the homeless.”

- 2.5 The consideration of uses falling under points A to D will rely on planning judgement on a case-by-case basis, having regard to the specific circumstances of the location and the wording in policy HH1. For hot food takeaways, the city council's adopted [Hot food takeaways supplementary planning document](#) (SPD) remains relevant in supporting the interpretation and implementation of relevant SLP:DMP policies. Policies HFTA1 and HFTA2 of the SPD are particularly relevant to this part of SLP:DMP policy HH1.

Development not in uses A) to D)

- 2.6 The applicant should set out how their scheme complies with policy HH1 with regards to supporting an improvement in public health and reducing health inequalities. In doing so account should be had to criteria 1 to 5 of policy HH1. It is likely that this justification will be in the planning statement or design and access statement.
- 2.7 To assist applicants in drafting their justification, a checklist has been prepared which shows how compliance with other relevant policies in the SLP:DMP (such as those in the design chapter) can contribute to addressing health and wellbeing issues. This checklist is at Annex 1 of this note. Regard should also be had to paragraph 14.4 of the SLP:DMP.
- 2.8 Smaller scale development may not warrant the submission of a planning statement / design and access statement. For such developments, Development Management case officers will still consider issues raised through policy HH1 to determine the potential for health impacts.

¹ A) Hot food takeaways; B) Off-licences; C) Facilities that encourage smoking, for example smoking shelters or shisha lounges; D) Payday lenders, betting shops and amusement arcades.

- 2.9 To determine whether any proposal potentially has a significant adverse impact on health and wellbeing the Development Management case officer will have regard to the following:
- Information submitted as part of the planning application process
 - Consultee comments such as those relating to air and noise
 - Annex 1 of this note
 - Paragraph 14.4 of the SLP:DMP
- 2.10 Development Management case officers should be able to reach a view if any significant health implications are likely to arise from a development. However, the council's Public Health and Spatial Planning teams will, where necessary, offer advice if particular health and wellbeing concerns have been identified.
- 2.11 If the Development Management case officer concludes that development will potentially have a significant adverse impact on health and wellbeing, policy HH1 requires the submission of a health impact assessment (HIA).
- 2.12 If insufficient information is submitted with an application, or not provided during the course of the application if requested, this could be a reason for refusal of a planning application.

Health Impact Assessments

- 2.13 It is likely that only a small number of developments will require HIA given that 'potential significant adverse impact' is a high threshold. Where HIA is considered to be necessary by the Development Management case officer, the applicant will be informed.
- 2.14 Where HIA is required, the requirements of criteria i) to iii) of policy HH1 will apply:
- i) Appraise the potential positive and negative impacts on health and wellbeing, both on end users and the local population;
 - ii) Consider the impacts on different groups, taking into account that some groups will be more vulnerable to negative impacts; and
 - iii) Set out actions to maximise positive impacts, and minimise and mitigate adverse impacts, on health and wellbeing, having regard to the most affected groups
- 2.15 Regard will also need to be had to the [Health Impact Assessment in spatial planning guide](#) produced by Public Health England. In preparing a HIA, applicants may find the [London Healthy Urban Development Unit Rapid HIA Tool](#) helpful.

3. Places for Everyone

- 3.1 [Places for Everyone](#) (PfE) is a plan of nine Greater Manchester districts (Bolton, Bury, Manchester, Oldham, Rochdale, Salford, Tameside, Trafford

and Wigan). It was adopted in March 2024 and forms part of the statutory development plan for Salford.

- 3.2 PfE policy JP-P6 Health part C requires that as part of helping to tackle health inequality, new development will be required as far as practicable to:

“Be supported by a Health Impact Assessment for all developments which require to be screened for an Environmental Impact Assessment, and other proposals which, due to their location, nature or proximity to sensitive receptors, are likely to have a notable impact on health and wellbeing”.

Implementation of PfE policy JP-P6: Health

Developments required to be screened for EIA

- 3.3 Developments which are required to be screened for Environmental Impact Assessment (EIA) are set out within the provisions of [The Town and Country Planning Environmental Impact Assessment Regulations 2017](#). Developments are typically those which are listed in Schedule 1 of these regulations or those listed in Schedule 2 which exceed the thresholds in column 2. Other development may also require screening if the site is environmentally sensitive. Such development will require a HIA to be prepared consistent with PfE policy JP-P6. The assessment shall have regard to parts i) to iii) of SLP:DMP policy HH1, and the documents referred to in paragraph 2.9 of this implementation note.

Other developments not screened for EIA

- 3.4 Where it has been concluded that a scheme does not have a significant adverse impact on health and well-being in the context of policy HH1 of the SLP:DMP, HIA will not be triggered by the requirements of JP-P6.

Annex 1: Development and health checklist

The checklist below has been adapted from that prepared by the [London Healthy Development Unit](#) (published in 2017). Its purpose is to assist applicants in demonstrating how development complies with the requirements of policy HH1 of the Salford Local Plan: Development Management Policies and Designations (SLP:DMP). In particular it sets out how development can support an improvement in public health and a reduction in health inequalities. It is of most relevance to those uses not listed in parts A) to D) of policy HH1.

Note that not all SLP:DMP policies listed below may be relevant to a scheme. Conversely, there may be other applicable SLP:DMP policies depending on the nature and location of the proposed development, such as the area policies set out in Chapter 8 of the plan. Specific policies in Places for Everyone may also be relevant.

Theme	Planning issue	Health and wellbeing issue	SLP:DMP policies in plan order
Healthy housing	- Housing design - Accessible housing - Healthy living - Housing mix and affordability	- Lack of living space - overcrowding - Unhealthy living environment – daylight, ventilation, noise - Excess deaths due to cold / overheating - Mental illness from social isolation and fear of crime	Policies F3, CC1, PC1, H1 to H10, TC5, HH1, WA4, WA5, D1, D3, D5, D6, D7, D9, G11, R1, PH1, and WM1.
Active travel	- Promoting walking and cycling - Safety - Connectivity - Minimising car use	- Physical inactivity, cardiovascular disease and obesity - Road and traffic injuries - Mental illness from social isolation - Noise and air pollution from traffic	Policies F3, CC1, PC1, TC1, TC3, H1, H3, H7, ED1, ED3, A1 to A13, D1, D4, D9, G11, R1, R3, R4 and R5.
Healthy environment	- Construction - Air quality - Noise - Contaminated land - Open space - Play space - Biodiversity	- Disturbance and stress caused by construction activity - Poor air quality - lung and heart disease - Disturbance from noisy activities and uses	Policies F3, CC1, EF1, PC1, TC5, HH1, A1, A3, A4, A5, A9, A10, EG1, EG2, WA3, WA4, WA5, D1, D3, D4, D5, D6, D9, GB1, G11 to G15, BG1, BG2, R1 to R5,

Theme	Planning issue	Health and wellbeing issue	SLP:DMP policies in plan order
	• Local food growing • Flood risk • Overheating	• Health risks from toxicity of contaminated land • Physical inactivity, cardiovascular disease and obesity • Mental health benefits from access to nature, green space, water and community facilities • Opportunities for food growing – active lifestyles, healthy diet and tackling food poverty • Excess summer deaths due to overheating	PH1 to PH4 and WM1.
Vibrant neighbourhoods	• Healthcare services • Education • Access to social infrastructure • Local employment and healthy workplaces • Access to local food shops • Public buildings and spaces	• Access to services and health inequalities • Mental illness and poor self-esteem associated with unemployment and poverty • Limited access to healthy food linked to obesity and related diseases • Poor environment leading to physical inactivity • Ill health exacerbated through isolation, lack of social contact and fear of crime	Policies F3, PC1, EC1, EC2, CT1, CT2, TC1 to TC5, ED1 to ED3, HH1 to HH3, A1, A2, A3, A7, A8, A10, DG1, DG2, D1, D4, D5, HE1, HE2, G11 to G14, BG1, BG2, and R1 to R5.